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THE 'FRUGAL' SURGEON

'All that glitters is not gold - All that is gold does not glitter'— J.R.R.Tolkien

An apt quote for what one goes through attending surgical conferences. They always showcase the new and upcoming innovations happening in the field. They are meant to dazzle as much as to educate. Young impressionable surgeons find themselves lured to shiny new instruments and technologies, some of which may not always be in the best interest of their patients and communities.

A patient chooses his/her surgeon and in most scenarios, not much else. The choice of surgery, technique, approach, instruments, materials, assistants, the operating room, the lights, the antibiotics, the analgesics, the hospital stay, and even the frequency of post op visits is made by the surgeon. It is endearing on the part of the patient to entrust the surgeon with all these choices. Therefore, it is of utmost significance that we exercise relentless caution and employ uniform and sound methodology in conducting our day to day practice.

One does not need multicentre studies to accept the deep tangible impact of high costs of cancer treatment. Financial toxicity and hospital free survival are relatively new terms which are a by-product of these costs. The mere existence of these terms are enough to prove the ill effects of modern medicine, similar to the latin term 'Res ipsa loquitur', meaning 'thing that speaks for itself'. A cancer surgeon has to walk a tightrope offering the best possible treatment while making sure that he/she does not indulge in any excesses. Identifying that thin line accurately where something of intrinsic value turns into an unnecessary expense is what goes into the making of a frugal surgeon. Frugality is a virtue, cost cutting is not. Cost cutting is when you compromise on patient safety and outcomes. Frugality is when you maintain a tight leash on unchecked consumerism. A

surgeon should always maintain his alliance with the patient and should resist becoming, albeit without realising, a vehicle for unfettered capitalism.

Being frugal is a journey that begins with the first visit of the patient and continues through long term follow up. Investigations to diagnose, stage and treat disease need to be evidence based. Every professional should shun and resist fancy, flashy, fashionable investigations and treatment modalities for miniscule benefits .

A few possible scenarios where one can introspect and discover opportunities to decrease the burden of cancer surgery on a daily basis are usage of staplers vs suturing in situations where both are possible -for instance, stapling Sigmoid colon before converting it into end stoma in abdominoperineal resection can be replaced by tying in two places and cutting in between. Use of Robotics only when a significant superiority is proven in comparison to conventional minimal access surgery is another example of the same.

It would be unfair to expect the patient to pay for the investment made by the hospital authorities apart from paying for his treatment. Hence we need to develop Institute policies with safety and affordability as a philosophy . This can be achieved by hyper-optimally utilising the infrastructure and man power so that the costs for patients stay lower and are distributed among many patients thus decreasing the burden on a few.

Another component of ensuring frugality is by preventing or minimising complications and hospitalization. This is ensured by well developed and meticulously adhered protocols and principles based on evidence and ERAS. Surgeons training themselves deeming it as a penance is the need of the hour-to become proficient in skill and decrease complications, blood transfusions ,stay and re explorations thus reducing or eliminating short and long term morbidity.

Data suggests that every year 6% of Indian population drops below poverty line because of unexpected

healthcare expenditure. The social impact of this is tremendous. Significant number of patients skip adjuvant treatment because of exhaustion of resources after surgery. Realization that the best results of cancer treatment are achieved by completion of treatment is important and Institutions should drive protocols to counsel the patients about multidisciplinary nature of treatment and stress on completing treatment. In the process, hospital needs to be committed to patients' benefits and divide the patients' resources to cover all the components of treatment to achieve cures. Surgeon needs to play a leader's role to ensure that the patient gets everything planned.

Institutionalization of care instead of independent practice can add to adherence to protocols and can aid participation in group negotiations for buying equipment and instruments which can increase the bargaining power of everyone and decrease costs for the hospitals and eventually the patients.

Eminent Surgeons in the country need to accept the responsibility to create a few 'Islands of Excellence ' where patients are treated correctly and completely irrespective of their financial comfort or otherwise. They should also ensure to prevent medicare administration being transferred to non medical managers which is significantly responsible for increasing hospital bills.

In the days of animated discussions on 'actionable and druggable' targets we should not neglect expanding 'operable' targets and tumours because the highest increment of benefit is by a good surgery that precedes or follows other therapies. In a world which is so complex simplicity is the name of the game and frugality is both a journey and a destination in this direction.

To conclude ,can we all say it is ideal to 'Frugalise and not Corporatize' cancer surgery because ...

"Without frugality none can be rich, and with it very few would be poor"— Samuel Johnson

